

CLINICAL NARRATIVE NOTE

PAT-1 | 4 January 2024

Patient Identification & Background

The patient registered under PAT-1 is a 41-year-old male, unmarried, and working as a farmer, who was registered under the unique patient identifier PAT-1. He belongs to the AB- blood group and has a prior reaction to sulfonamides presenting with urticaria. The patient is a smoker of approximately ten cigarettes per day with occasional alcohol consumption. He presented to the outpatient department on 4 January 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was recurrent epigastric pain and heartburn despite self-medicating with antacids. He has a known background of symmetrical joint pain involving both hands and wrists for over three years, with recent DEXA scan confirming reduced bone density. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Rheumatoid Arthritis and Osteoporosis, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 110/70 mmHg, heart rate was 62 beats per minute, respiratory rate was 15 breaths per minute, and body temperature was 97.8°F. Oxygen saturation on room air was 100%. The patient's recorded weight was 55 kg with a height of 156 cm, yielding a body mass index of 22.6 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. He appeared in mild distress. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included synovitis of MCP and PIP joints bilaterally with tenderness on palpation. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated elevated RF titre of 1:160 and anti-CCP antibody positive. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Warfarin 5 mg once daily (INR monitoring monthly), Carvedilol 6.25 mg twice daily, Furosemide 40 mg once daily, and Spironolactone 25 mg once

daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

The clinical evaluation today suggests that PAT-1 remains showing gradual improvement with appropriate management. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears moderately adherent and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Rheumatoid Arthritis and Osteoporosis. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, current medications are maintained without changes as the patient remains well controlled. Furthermore, dietary and lifestyle counselling was reinforced at this visit. He was also informed that annual screening investigations have been ordered as per protocol. Finally, the patient is to be reviewed in three months and instructed to seek earlier review if any new symptoms arise. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 4 January 2024 | PAT-1

CLINICAL NARRATIVE NOTE

PAT-2 | 3 February 2024

Patient Identification & Background

The patient registered under PAT-2 is a 44-year-old male, divorced, and working as an accountant, who was registered under the unique patient identifier PAT-2. He belongs to the A- blood group and has intolerance to codeine with nausea and vomiting. The patient is a non-smoker and does not consume alcohol. He presented to the outpatient department on 3 February 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was poorly controlled blood pressure with home readings consistently above 150/95 mmHg. He has a known background of recurrent unilateral throbbing headaches with photophobia and nausea occurring 3–4 times per month, along with generalised anxiety impacting daily functioning. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Migraine without Aura and Anxiety Disorder, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 118/74 mmHg, heart rate was 68 beats per minute, respiratory rate was 16 breaths per minute, and body temperature was 98.2°F. Oxygen saturation on room air was 99%. The patient's recorded weight was 61 kg with a height of 160 cm, yielding a body mass index of 23.8 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. He appeared in mild distress. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included no focal neurological deficits; visual fields intact. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated neuroimaging unremarkable; GAD-7 score of 16. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Pantoprazole 40 mg once daily before breakfast, Sucralfate 1 g four times daily, Domperidone 10 mg three times daily before meals, and Helicobacter pylori eradication therapy completed. Medication reconciliation was performed at today's visit,

and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

The clinical evaluation today suggests that PAT-2 remains clinically stable with no significant deterioration. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears moderately adherent and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Migraine without Aura and Anxiety Disorder. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, medications are continued at the current doses with a plan to reassess at the next visit. Furthermore, repeat laboratory investigations have been ordered including complete blood count, renal function tests, and lipid profile. He was also informed that the patient has been counselled extensively on dietary modifications, regular physical activity, and the importance of medication compliance. Finally, a follow-up appointment has been scheduled in eight weeks or sooner if symptoms worsen. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 3 February 2024 | PAT-2

CLINICAL NARRATIVE NOTE

PAT-3 | 3 March 2024

Patient Identification & Background

The patient registered under PAT-3 is a 47-year-old female, unmarried, and working as a nurse, who was registered under the unique patient identifier PAT-3. She belongs to the AB- blood group and has intolerance to codeine with nausea and vomiting. The patient is an ex-smoker who quit five years ago with no current alcohol use. She presented to the outpatient department on 3 March 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was worsening joint pains in both hands with early morning stiffness lasting more than an hour. She has a known background of recurrent episodes of wheezing, breathlessness, and nasal congestion, particularly worsening in cold weather. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Bronchial Asthma and Allergic Rhinitis, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 122/78 mmHg, heart rate was 72 beats per minute, respiratory rate was 17 breaths per minute, and body temperature was 98.4°F. Oxygen saturation on room air was 99%. The patient's recorded weight was 65 kg with a height of 163 cm, yielding a body mass index of 24.5 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was alert and well-oriented. General examination showed pallor in the conjunctivae. Cardiovascular examination demonstrated normal heart sounds without murmurs. Chest auscultation was clear. Peripheral pulsations were well felt. Pertinent examination findings relevant to the presenting diagnosis included bilateral expiratory rhonchi on auscultation. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated peak flow readings consistently below 75% of predicted value. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Rosuvastatin 20 mg at bedtime, Aspirin 75 mg once daily, Clopidogrel 75 mg once daily, and Metoprolol succinate 50 mg once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed

regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

On today's assessment, PAT-3 is managing well overall despite the chronicity of the underlying conditions. There is notable improvement since the last visit, and the patient has been largely compliant regarding medication compliance and lifestyle recommendations. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Bronchial Asthma and Allergic Rhinitis. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the dose of the primary medication has been uptitrated in view of suboptimal control. Furthermore, urgent referral to the relevant specialist has been initiated. She was also informed that patient education regarding warning signs requiring emergency consultation was provided. Finally, repeat investigations have been ordered and the patient is to return in four weeks for review. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 3 March 2024 | PAT-3

CLINICAL NARRATIVE NOTE

PAT-4 | 2 April 2024

Patient Identification & Background

The patient registered under PAT-4 is a 50-year-old male, divorced, and working as a daily wage labourer, who was registered under the unique patient identifier PAT-4. He belongs to the A- blood group and has intolerance to codeine with nausea and vomiting. The patient is an ex-smoker who quit five years ago with no current alcohol use. He presented to the outpatient department on 2 April 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was generalised body weakness, poor appetite, and unintentional weight loss of approximately 4 kg over two months. He has a known background of fatigue, weight gain, and depressive symptoms over the past six months, initially attributed to mood disorder before thyroid workup confirmed hypothyroidism. The symptoms have been present and progressing over recent weeks, prompting he to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Hypothyroidism and Depression, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 136/86 mmHg, heart rate was 80 beats per minute, respiratory rate was 19 breaths per minute, and body temperature was 98.8°F. Oxygen saturation on room air was 98%. The patient's recorded weight was 78 kg with a height of 170 cm, yielding a body mass index of 27.0 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was comfortable at rest but mildly dyspnoeic on exertion. Pulse was irregular. Lung bases showed fine crepitations. Bilateral pitting pedal oedema of grade 2 was noted. Liver was palpable 2 cm below the right costal margin. Pertinent examination findings relevant to the presenting diagnosis included mild facial puffiness and slow relaxing deep tendon reflexes. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated TSH of 8.9 mIU/L and free T4 of 0.7 ng/dL. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Tiotropium 18 mcg inhaler once daily, Salbutamol 100 mcg inhaler PRN, CPAP therapy 10 cmH₂O nightly, and N-acetylcysteine 600 mg twice

daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

On today's assessment, PAT-4 is managing well overall despite the chronicity of the underlying conditions. There is marginal but encouraging progress since the last visit, and the patient has been well adherent and motivated regarding medication compliance and lifestyle recommendations. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Hypothyroidism and Depression. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, a new medication has been added to the existing regimen after discussing the rationale and potential side effects with the patient. Furthermore, physiotherapy referral has been initiated for symptomatic relief. He was also informed that vaccination status was reviewed and influenza vaccine administered. Finally, follow-up in six weeks with repeat biochemical parameters. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 2 April 2024 | PAT-4

CLINICAL NARRATIVE NOTE

PAT-5 | 2 May 2024

Patient Identification & Background

The patient registered under PAT-5 is a 53-year-old female, unmarried, and working as a business owner, who was registered under the unique patient identifier PAT-5. She belongs to the AB- blood group and has latex sensitivity with contact dermatitis. The patient is a non-smoker and does not consume alcohol. She presented to the outpatient department on 2 May 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was persistent headaches occurring almost daily for the past two weeks. She has a known background of recurrent unilateral throbbing headaches with photophobia and nausea occurring 3–4 times per month, along with generalised anxiety impacting daily functioning. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Migraine without Aura and Anxiety Disorder, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 118/74 mmHg, heart rate was 68 beats per minute, respiratory rate was 16 breaths per minute, and body temperature was 98.2°F. Oxygen saturation on room air was 99%. The patient's recorded weight was 61 kg with a height of 160 cm, yielding a body mass index of 23.8 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was well-built and cooperative with no acute distress. Vital signs were within acceptable range. Neurological examination was grossly intact. Deep tendon reflexes were normal and symmetrical. No focal neurological deficits identified. Pertinent examination findings relevant to the presenting diagnosis included no focal neurological deficits; visual fields intact. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated neuroimaging unremarkable; GAD-7 score of 16. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Salbutamol 100 mcg inhaler two puffs as needed, Budesonide/Formoterol 160/4.5 mcg inhaler twice daily, Montelukast 10 mg at bedtime, and Cetirizine 10 mg once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. She was counselled regarding the importance of not self-

discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

On today's assessment, PAT-5 is suboptimally controlled and requiring modification of the current regimen. There is some deterioration requiring attention since the last visit, and the patient has been well adherent and motivated regarding medication compliance and lifestyle recommendations. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Migraine without Aura and Anxiety Disorder. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, medications are continued at the current doses with a plan to reassess at the next visit. Furthermore, repeat laboratory investigations have been ordered including complete blood count, renal function tests, and lipid profile. She was also informed that the patient has been counselled extensively on dietary modifications, regular physical activity, and the importance of medication compliance. Finally, a follow-up appointment has been scheduled in eight weeks or sooner if symptoms worsen. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 2 May 2024 | PAT-5

CLINICAL NARRATIVE NOTE

PAT-6 | 1 June 2024

Patient Identification & Background

The patient registered under PAT-6 is a 56-year-old male, divorced, and working as a retired government officer, who was registered under the unique patient identifier PAT-6. He belongs to the A- blood group and has latex sensitivity with contact dermatitis. The patient is a non-smoker and does not consume alcohol. He presented to the outpatient department on 1 June 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was recurrent episodes of breathlessness on climbing stairs or walking more than 200 metres. He has a known background of recurrent episodes of wheezing, breathlessness, and nasal congestion, particularly worsening in cold weather. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Bronchial Asthma and Allergic Rhinitis, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 128/82 mmHg, heart rate was 76 beats per minute, respiratory rate was 18 breaths per minute, and body temperature was 98.4°F. Oxygen saturation on room air was 98%. The patient's recorded weight was 72 kg with a height of 168 cm, yielding a body mass index of 25.5 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was mildly anxious but cooperative. Skin and mucous membranes were moist. Thyroid was not palpably enlarged. Cardiovascular and respiratory examinations were unremarkable. Abdominal examination revealed no organomegaly. Pertinent examination findings relevant to the presenting diagnosis included bilateral expiratory rhonchi on auscultation. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated peak flow readings consistently below 75% of predicted value. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Methotrexate 15 mg once weekly, Folic acid 5 mg once weekly (day after methotrexate), Hydroxychloroquine 200 mg twice daily, and Alendronate 70 mg once weekly. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing

medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

Overall, the patient's condition appears to be showing gradual improvement with appropriate management. The clinical picture is consistent with the underlying diagnoses, and the patient demonstrates occasionally non-compliant due to side effects to the prescribed treatment plan. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Bronchial Asthma and Allergic Rhinitis. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the dose of the primary medication has been uptitrated in view of suboptimal control. Furthermore, urgent referral to the relevant specialist has been initiated. He was also informed that patient education regarding warning signs requiring emergency consultation was provided. Finally, repeat investigations have been ordered and the patient is to return in four weeks for review. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 1 June 2024 | PAT-6

CLINICAL NARRATIVE NOTE

PAT-7 | 1 July 2024

Patient Identification & Background

The patient registered under PAT-7 is a 59-year-old female, unmarried, and working as a homemaker, who was registered under the unique patient identifier PAT-7. She belongs to the AB- blood group and has latex sensitivity with contact dermatitis. The patient is an ex-smoker who quit five years ago with no current alcohol use. She presented to the outpatient department on 1 July 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was worsening fatigue and increased thirst over the past three weeks. She has a known background of fatigue, weight gain, and depressive symptoms over the past six months, initially attributed to mood disorder before thyroid workup confirmed hypothyroidism. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Hypothyroidism and Depression, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 136/86 mmHg, heart rate was 80 beats per minute, respiratory rate was 19 breaths per minute, and body temperature was 98.8°F. Oxygen saturation on room air was 98%. The patient's recorded weight was 78 kg with a height of 170 cm, yielding a body mass index of 27.0 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was mildly anxious but cooperative. Skin and mucous membranes were moist. Thyroid was not palpably enlarged. Cardiovascular and respiratory examinations were unremarkable. Abdominal examination revealed no organomegaly. Pertinent examination findings relevant to the presenting diagnosis included mild facial puffiness and slow relaxing deep tendon reflexes. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated TSH of 8.9 mIU/L and free T4 of 0.7 ng/dL. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Metformin 500 mg twice daily with meals, Lisinopril 10 mg once daily in the morning, Aspirin 81 mg once daily, and Atorvastatin 20 mg at bedtime.

Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

Overall, the patient's condition appears to be clinically stable with no significant deterioration. The clinical picture is consistent with the underlying diagnoses, and the patient demonstrates moderately adherent to the prescribed treatment plan. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Hypothyroidism and Depression. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, a new medication has been added to the existing regimen after discussing the rationale and potential side effects with the patient. Furthermore, physiotherapy referral has been initiated for symptomatic relief. She was also informed that vaccination status was reviewed and influenza vaccine administered. Finally, follow-up in six weeks with repeat biochemical parameters. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 1 July 2024 | PAT-7

CLINICAL NARRATIVE NOTE

PAT-8 | 28 August 2024

Patient Identification & Background

The patient registered under PAT-8 is a 62-year-old female, divorced, and working as a school teacher, who was registered under the unique patient identifier PAT-8. She belongs to the A- blood group and has NSAID hypersensitivity causing gastrointestinal bleeding. The patient is a non-smoker and does not consume alcohol. She presented to the outpatient department on 28 August 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was routine follow-up for management of existing chronic conditions with no new acute complaints. She has a known background of persistent heartburn, regurgitation, and epigastric pain worsening post-meals and on lying down. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Gastroesophageal Reflux Disease and Peptic Ulcer Disease, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 155/95 mmHg, heart rate was 94 beats per minute, respiratory rate was 22 breaths per minute, and body temperature was 99.1°F. Oxygen saturation on room air was 96%. The patient's recorded weight was 93 kg with a height of 175 cm, yielding a body mass index of 30.4 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. He appeared in mild distress. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included mild epigastric tenderness on deep palpation. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated upper GI endoscopy revealing grade B esophagitis and a 1.2 cm antral ulcer. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Levothyroxine 75 mcg once daily on empty stomach, Sertraline 50 mg once daily, Calcium carbonate 500 mg twice daily, and Melatonin 5 mg at bedtime. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

The clinical evaluation today suggests that PAT-8 remains clinically stable with no significant deterioration. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears moderately adherent and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Gastroesophageal Reflux Disease and Peptic Ulcer Disease. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, current medications are maintained without changes as the patient remains well controlled. Furthermore, dietary and lifestyle counselling was reinforced at this visit. She was also informed that annual screening investigations have been ordered as per protocol. Finally, the patient is to be reviewed in three months and instructed to seek earlier review if any new symptoms arise. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 28 August 2024 | PAT-8

CLINICAL NARRATIVE NOTE

PAT-9 | 28 September 2024

Patient Identification & Background

The patient registered under PAT-9 is a 65-year-old male, unmarried, and working as a software engineer, who was registered under the unique patient identifier PAT-9. He belongs to the AB- blood group and has NSAID hypersensitivity causing gastrointestinal bleeding. The patient is a non-smoker and does not consume alcohol. He presented to the outpatient department on 28 September 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was recurrent epigastric pain and heartburn despite self-medicating with antacids. He has a known background of progressive renal impairment noted over the past two years alongside persistently elevated blood pressure. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Chronic Kidney Disease Stage 3 and Hypertension, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 128/82 mmHg, heart rate was 76 beats per minute, respiratory rate was 18 breaths per minute, and body temperature was 98.4°F. Oxygen saturation on room air was 98%. The patient's recorded weight was 72 kg with a height of 168 cm, yielding a body mass index of 25.5 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was alert and well-oriented. General examination showed pallor in the conjunctivae. Cardiovascular examination demonstrated normal heart sounds without murmurs. Chest auscultation was clear. Peripheral pulsations were well felt. Pertinent examination findings relevant to the presenting diagnosis included mild pitting edema in the lower extremities. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated serum creatinine of 2.1 mg/dL and eGFR of 42 mL/min/1.73m². These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Sumatriptan 50 mg at onset of migraine (max 2 doses/24h), Propranolol 40 mg twice daily for prophylaxis, Escitalopram 10 mg once daily, and Clonazepam 0.5 mg at bedtime PRN. Medication reconciliation was performed at today's visit, and the

patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

The clinical evaluation today suggests that PAT-9 remains managing well overall despite the chronicity of the underlying conditions. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears largely compliant and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Chronic Kidney Disease Stage 3 and Hypertension. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, medications are continued at the current doses with a plan to reassess at the next visit. Furthermore, repeat laboratory investigations have been ordered including complete blood count, renal function tests, and lipid profile. He was also informed that the patient has been counselled extensively on dietary modifications, regular physical activity, and the importance of medication compliance. Finally, a follow-up appointment has been scheduled in eight weeks or sooner if symptoms worsen. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 28 September 2024 | PAT-9

CLINICAL NARRATIVE NOTE

PAT-10 | 27 October 2024

Patient Identification & Background

The patient registered under PAT-10 is a 68-year-old female, divorced, and working as a shop keeper, who was registered under the unique patient identifier PAT-10. She belongs to the A- blood group and has NSAID hypersensitivity causing gastrointestinal bleeding. The patient is a smoker of approximately ten cigarettes per day with occasional alcohol consumption. She presented to the outpatient department on 27 October 2024 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness

The patient's primary reason for the visit today was worsening joint pains in both hands with early morning stiffness lasting more than an hour. She has a known background of exertional dyspnea and orthopnea with documented reduced ejection fraction of 38% and persistent atrial fibrillation. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses

The patient carries an established diagnosis of Heart Failure with Reduced EF and Atrial Fibrillation, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements

On examination today, vital signs were recorded as follows: blood pressure was 148/92 mmHg, heart rate was 92 beats per minute, respiratory rate was 21 breaths per minute, and body temperature was 99.0°F. Oxygen saturation on room air was 97%. The patient's recorded weight was 88 kg with a height of 174 cm, yielding a body mass index of 29.1 kg/m², which is noted for ongoing monitoring.

Physical Examination

On general inspection, the patient was comfortable at rest but mildly dyspnoeic on exertion. Pulse was irregular. Lung bases showed fine crepitations. Bilateral pitting pedal oedema of grade 2 was noted. Liver was palpable 2 cm below the right costal margin. Pertinent examination findings relevant to the presenting diagnosis included irregularly irregular pulse, basal crepitations in both lung bases. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings

Relevant investigations reviewed at this visit demonstrated BNP of 620 pg/mL and mildly elevated troponin I. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications

The patient is currently prescribed the following medications: Rosuvastatin 20 mg at bedtime, Aspirin 75 mg once daily, Clopidogrel 75 mg once daily, and Metoprolol succinate 50 mg once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed

regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment

On today's assessment, PAT-10 is suboptimally controlled and requiring modification of the current regimen. There is some deterioration requiring attention since the last visit, and the patient has been well adherent and motivated regarding medication compliance and lifestyle recommendations. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Heart Failure with Reduced EF and Atrial Fibrillation. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up

Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the dose of the primary medication has been uptitrated in view of suboptimal control. Furthermore, urgent referral to the relevant specialist has been initiated. She was also informed that patient education regarding warning signs requiring emergency consultation was provided. Finally, repeat investigations have been ordered and the patient is to return in four weeks for review. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 27 October 2024 | PAT-10